

D399

Introduction to Gerontology

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 78-year-old retired engineer who still does crossword puzzles daily has twice left the stove on and got lost driving to his usual grocery store this month. His wife says he has always been forgetful about names. What is the best interpretation?

- A. Normal aging, since he still does puzzles.
- B. Normal, since he has always forgotten names.
- C. These changes are not normal aging and should be evaluated.
- D. Normal aging, because all older adults become forgetful.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

An 84-year-old woman in assisted living was chatting normally two days ago. Since yesterday she has been drowsy one hour and agitated the next and cannot follow a conversation. She was just started on a new bladder medication. She has three great-grandchildren. What is most likely?

- A. Depression, because she is far from her family.
- B. Dementia that has suddenly appeared.
- C. Normal aging.
- D. Delirium, possibly from the new medication, needing prompt evaluation.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

Which history best fits dementia rather than delirium?

- A. Drowsy in the morning and agitated at night since a new drug.
- B. Any confusion in someone over 85.
- C. Slowly worsening memory and trouble managing bills over two years, with normal alertness.
- D. Confusion that started last night during a urinary infection.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 79-year-old man takes nine medicines. After starting diphenhydramine (a sedating antihistamine) for sleep, he becomes confused and falls. He lives alone with a small dog. What should be done first?

- A. Increase the dose so he sleeps through the night.
- B. Review his medications and stop or replace the diphenhydramine.
- C. Add a medication for confusion.
- D. Rehome the dog to prevent falls.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 76-year-old woman fell twice last year. She takes a blood pressure pill, has cataracts, and has loose rugs at home. She is a former high school principal. Which plan best reduces her risk?

- A. Give her a cane.
- B. Combine balance exercise, a medication and orthostatic blood pressure check, eye care, and removing rugs.
- C. Nothing; falls are an expected part of aging.
- D. Tell her to stay in bed to avoid falls.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A home care aide notices that an 81-year-old client's bank statements show large withdrawals since her nephew moved in. The nephew now answers every question for her. Her home is tidy and she wears clean clothes. What should the aide do?

- A. Report the suspected financial exploitation to Adult Protective Services.
- B. Wait until there is proof.
- C. Confront the nephew.
- D. Do nothing, since she looks well cared for.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

After a hip fracture and a four-day hospital stay, an 88-year-old woman goes to a skilled nursing facility. Her family has just repainted her house. She now needs help with bathing and dressing indefinitely but no skilled care. Which statement about payment is true?

- A. Medicare pays for her nursing home stay for as long as she needs it.
- B. Medicare pays for custodial care, and Medicaid pays for skilled rehab.
- C. The Older Americans Act pays nursing home costs.
- D. Medicare will not pay for indefinite custodial care; Medicaid may if she qualifies financially.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

An older man living at home needs help getting meals and connecting to local services. Which program is designed to fund this?

- A. Social Security Disability Insurance.
- B. Medicare Part A.
- C. The Older Americans Act, through Area Agencies on Aging.
- D. Medicaid only, and only in nursing homes.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A 90-year-old woman with mild hearing loss comes to the clinic with her son, who is a lawyer. The staff member addresses the son and calls the woman “sweetie.” The clinic is busy with music playing. Which change best supports her dignity?

- A. Keep talking to the son, since he is a lawyer.
- B. Face her, speak to her directly by her preferred name at a lower pitch, and turn down the music.
- C. Use written notes only from now on.
- D. Speak louder and slower to her in simple words.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

Which statement is an example of elderspeak?

- A.** “How are we feeling today, honey?”
- B.** “Would you like me to repeat that?”
- C.** “I’ll sit here so you can see my face.”
- D.** “Mrs. Ortiz, how are you feeling today?”

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

An 83-year-old man with early Alzheimer’s disease, who was a carpenter for 50 years, understands the risks and says he wants to stay in his home rather than move to assisted living. His daughter insists he move. What guides the team?

- A.** He should stay home because he can fix things.
- B.** The team decides what is safest.
- C.** The daughter decides because he has Alzheimer’s.
- D.** If he has capacity for this decision, his choice is respected, with supports to reduce risk.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

What best describes decision-making capacity?

- A.** The ability to understand, weigh, and communicate a choice about a specific decision.
- B.** A legal ruling made by a judge.
- C.** Determined by age.
- D.** Lost for all decisions once dementia is diagnosed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A 78-year-old retired engineer ~~who still does crossword puzzles daily~~ has twice left the stove on and got lost driving to his usual grocery store this month. ~~His wife says he has always been forgetful about names.~~ What is the best interpretation?

Target word: normal vs. not normal **Decides:** Getting lost on a familiar route and safety lapses disrupt daily life.

A	Camouflage	Puzzle skill does not rule out impairment.
B	Confusion set	Lifelong name forgetting differs from new safety problems.
C	Correct	Getting lost on familiar routes is a warning sign.
D	Absolute language	Mild forgetfulness is normal; disruptive loss is not.

2. Answer D Concept 2

An 84-year-old woman in assisted living was chatting normally two days ago. Since yesterday she has been drowsy one hour and agitated the next and cannot follow a conversation. She was just started on a new bladder medication. ~~She has three great grandchildren.~~ What is most likely?

Target word: delirium vs. dementia **Decides:** Sudden onset, fluctuating attention, new drug.

A	Camouflage	Invents a cause from the family detail.
B	Confusion set	Dementia develops gradually.
C	Adjacent category	Sudden confusion is never normal aging.
D	Correct	Acute, fluctuating, and often reversible.

3. Answer C Concept 2 · transfer

Which history best fits dementia rather than delirium?

Target word: reverse: pick the dementia pattern **Decides:** Gradual decline over time.

A	Confusion set	Fluctuation after a drug change is delirium.
B	Absolute language	Age alone does not decide.
C	Correct	Gradual onset with intact attention fits dementia.
D	Inverted operation	Acute onset with infection is delirium.

4. Answer B Concept 3

A 79-year-old man takes nine medicines. After starting diphenhydramine (a sedating antihistamine) for sleep, he becomes confused and falls. ~~He lives alone with a small dog.~~ What should be done first?

Target word: first step **Decides:** New symptom after an anticholinergic drug.

A	Absolute language	More of the harmful drug.
B	Correct	It is on the Beers list for older adults.
C	Inverted operation	Treats a side effect with another drug.
D	Camouflage	Seizes on the dog; the new drug is the likely cause.

5. Answer B Concept 4

A 76-year-old woman fell twice last year. She takes a blood pressure pill, has cataracts, and has loose rugs at home. ~~She is a former high school principal.~~ Which plan best reduces her risk?

Target word: best prevention approach **Decides:** Multiple risk factors need multiple fixes.

A	Dropped element	Addresses only one factor.
B	Correct	Multifactorial prevention works best.
C	Absolute language	Falls are preventable.
D	Inverted operation	Inactivity worsens strength and fall risk.

6. Answer A Concept 5

A home care aide notices that an 81-year-old client's bank statements show large withdrawals since her nephew moved in. The nephew now answers every question for her. ~~Her home is tidy and she wears clean clothes.~~ What should the aide do?

Target word: response to suspicion **Decides:** Signs of financial exploitation and control.

A	Correct	Suspicion is enough to report.
B	Dropped element	Reporters need reasonable suspicion, not proof.
C	Adjacent category	May put her at risk; reporting is the proper step.
D	Camouflage	A tidy home does not rule out financial abuse.

7. Answer D Concept 6

After a hip fracture and a four-day hospital stay, an 88-year-old woman goes to a skilled nursing facility. ~~Her family has just repainted her house.~~ She now needs help with bathing and dressing indefinitely but no skilled care. Which statement about payment is true?

Target word: payer for long-term custodial care **Decides:** Medicare covers only short-term skilled care.

A	Absolute language	Medicare's SNF benefit is limited and skilled-only.
B	Inverted operation	Reverses the two programs' roles.
C	Confusion set	It funds community services like meals and caregiver support.
D	Correct	Medicaid is the main public payer for long-term care.

8. Answer C Concept 6 • transfer

An older man living at home needs help getting meals and connecting to local services. Which program is designed to fund this?

Target word: reverse: from service to program **Decides:** Community services for older adults.

A	Confusion set	Income support, not services.
B	Adjacent category	Hospital insurance, not meals.
C	Correct	Funds home-delivered meals and supportive services.
D	Absolute language	Medicaid does fund some home services, and the Older Americans Act is the main meals program.

9. Answer B Concept 7

A 90-year-old woman with mild hearing loss comes to the clinic with her son, ~~who is a lawyer~~. The staff member addresses the son and calls the woman “sweetie.” The clinic is busy with music playing. Which change best supports her dignity?

Target word: respectful communication **Decides:** Talking past her and elderspeak.

A	Camouflage	His job does not make him the patient.
B	Correct	Direct, respectful, hearing-friendly communication.
C	Absolute language	Mild hearing loss does not rule out speaking.
D	Adjacent category	Shouting and simplified talk are elderspeak.

10. Answer A Concept 7 · transfer

Which statement is an example of elderspeak?

Target word: reverse: identify the violation **Decides:** Patronizing tone and terms.

A	Correct	Collective “we” and pet names are elderspeak.
B	Confusion set	Checking understanding is good practice.
C	Adjacent category	A hearing-loss strategy, not elderspeak.
D	Inverted operation	Respectful, direct address.

11. Answer D Concept 8

An 83-year-old man with early Alzheimer’s disease, ~~who was a carpenter for 50 years~~, understands the risks and says he wants to stay in his home rather than move to assisted living. His daughter insists he move. What guides the team?

Target word: who decides **Decides:** He shows capacity for this decision.

A	Camouflage	His trade does not decide capacity.
B	Inverted operation	Replaces his autonomy with the team’s.
C	Absolute language	A diagnosis does not remove all decision rights.
D	Correct	Capacity is decision-specific; a diagnosis doesn’t remove it.

12. Answer A Concept 8 · transfer

What best describes decision-making capacity?

Target word: reverse: define the concept **Decides:** Decision-specific and assessed.

A	Correct	Capacity is task-specific.
B	Confusion set	That is competence, a legal term.
C	Adjacent category	Age alone never decides capacity.
D	Absolute language	Capacity can remain for many decisions.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	2	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	6	
Absolute language	Always, never, guarantee — where judgment applies.	9	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Normal aging vs. disease	Mild slowing, occasional word-finding pauses, and needing reading glasses are normal aging; memory loss that disrupts daily life, new confusion, or incontinence is not normal and needs evaluation.	Dismissing new problems as “just old age.”	An older adult’s change: gradual and minor vs. disruptive or new.
2	Delirium vs. dementia	Delirium starts suddenly, fluctuates, impairs attention, and is often reversible once the cause (infection, medication, dehydration) is treated; dementia develops gradually over months to years.	Assuming sudden confusion in an older adult is dementia.	Onset measured in hours or days vs. months or years.
3	Polypharmacy and the Beers Criteria	Older adults are more sensitive to many drugs; the Beers Criteria flag drugs to avoid, such as strong anticholinergics and benzodiazepines, and every new symptom should prompt a medication review.	Treating a side effect with another drug instead of reviewing the list.	Several medications and a new symptom such as falls or confusion.
4	Fall risk is multifactorial	Falls are the leading cause of injury death in adults 65 and older; prevention combines strength and balance exercise, medication review, vision checks, orthostatic blood pressure, and home hazard fixes.	Relying on one fix, such as a cane, or on restricting activity.	A fall history or a question about the best prevention approach.
5	Elder abuse recognition and reporting	Elder abuse includes physical, emotional, sexual, neglect, and financial exploitation; suspected abuse is reported to Adult Protective Services even without proof.	Thinking only bruises count, or that proof is needed before reporting.	Unexplained money changes, isolation, or a caregiver who answers for the older adult.
6	Who pays for long-term care	Medicare covers short-term skilled care after a qualifying hospital stay but not long-term custodial care; Medicaid is the main public payer for long-term nursing home care; the Older Americans Act funds community services like meals and Area Agencies on Aging.	Believing Medicare pays for an indefinite nursing home stay.	Custodial care, home-delivered meals, or spending down assets.

#	Concept	Rule	Trap	Tell
7	Dignified communication	Speak directly to the older adult in normal adult language; avoid “elderspeak”; for hearing loss face the person, lower pitch, and reduce background noise instead of shouting.	Talking to the family instead of the person, or shouting and using pet names.	A companion present, hearing loss, or terms like “sweetie.”
8	Capacity and autonomy	An older adult with decision-making capacity has the right to make choices others disagree with; capacity is decision-specific and assessed, not assumed from age or diagnosis.	Letting family override a capable older adult, or assuming a dementia diagnosis removes all decision rights.	Family disagreement with an older adult’s choice.

Course scope (public description): Introduction to Gerontology will introduce learners to health issues that are typically associated with the older adult population so they can become familiar with health challenges this population may face. The learners will gain an understanding of the effects that policy and legislation have on the older adult population. Emphasis is placed on the importance of maintaining the dignity of older adults by focusing on cultural and communication needs, and by collaborating on care with older adults, families, and caregivers. There are no prerequisites for this course.